

DOCUMENT METADATA

Document Name:

NOSI 9-3-24 Women Veteran's Health and Health Care.pdf

Title:

NOSI 9-3-24 Women Veteran's Health and Health Care

Topic:

Women's Health – Maternity and Infertility

Document Type:

Presentations and Other Documents

Created:

2024-09-20 by Richman, Michael (VACO)

Modified:

2026-02-02 by Bartlinski, David



**Office of Research and Development
Investigators, Scientific Review and Management (ISRM)**

**Notice of Special Interest:
Women Veteran's Health and Healthcare
NOT-RD-01-WMH**

Release Date: September 3, 2024

For Research Application Submissions: All Cycles

Expiration Date: Not applicable.

Applications will be accepted through this Notice of Special Interest (NOSI) to the following RFAs:

- RD-01-MRA ISRM Parent Merit Review Award (Non-Clinical Trials/I01)
- RD-01-MRCT ISRM Parent Merit Review Award (Clinical Trials/I01)
- RD-01-PILT ISRM Pilot Project Award (I21)
- RD-01-CDA1 ISRM Career Development Award (CDA-1) (IK1)
- RD-01-CDA2 ISRM Career Development Award (CDA-2) (IK2)

Participating VA-ORD Research Portfolios:

Health Systems Research Broad Portfolio (BP), Portfolio Identifier = HSR
Rehabilitation Research, Development and Translation BP, Portfolio Identifier = RDT
Medical Health Research BP, Portfolio Identifier = MED
Brain, Behavioral and Mental Health BP, Portfolio Identifier = BMH
Suicide Prevention Actively Managed Portfolio (AMP), Portfolio Identifier = SPR
Traumatic Brain Injury AMP, Portfolio Identifier = TBI
Military Exposures Research AMP, Portfolio Identifier = MER
Program: 1990-91 Gulf War Research, Portfolio Identifier = GWR

Purpose: The Veterans Health Administration (VHA), Office of Research and Development (VA-ORD) releases this funding opportunity announcement for studies on improving the health and healthcare of women Veterans across the lifespan, including in health areas that affect women Veterans uniquely, disproportionately, or differently from men.

Background: Women are the fastest growing group in the Veteran population,ⁱ and women Veterans between the ages of 45-64 years old using VHA has grown substantially.ⁱⁱ These shifts are changing the demographics of the Veterans served and the range of services VA providers must be prepared to deliver. Women Veterans who use the VA are as complex medically as their male counterparts, even though they are on average 10 years younger. Women Veterans present with complex physical and co-occurring mental health-related multi-morbidities,ⁱⁱⁱ this

could be partially due to much higher rates of military sexual trauma (MST) and other mental health disorders. The need to deliver gender-specific care (e.g., reproductive health services) and care for conditions of higher prevalence (e.g., osteoporosis, depression) or with different clinical presentations (e.g., heart disease, chronic pain) among women also imposes considerable training and experiential requirements on a VA workforce with historically limited exposure to women patients.ⁱⁱ Women Veterans also need medical equipment and components designed and tailored to them, including prosthetic components. While there have been advances in VA women's care, women may still face barriers and challenges related to access, continuity of care, and care coordination.ⁱⁱⁱ This can lead to attrition and discontinuity of care or seeking community or other non-VA care at higher rates.

In March 2024, President Biden signed Executive Order (EO) 14120 – Advancing Women's Health Research and Innovation. This EO is designed to "...accelerate research that will provide the tools we need to prevent, diagnose, and treat conditions that affect women uniquely, disproportionately, or differently." The VA is tasked with funding research that is responsive to the priorities in the EO.

This Notice of Special Interest (NOSI) proposes a research agenda to improve the health and healthcare of women Veterans across the lifespan.

Overarching Research Priorities: Proposed women Veteran's health studies must be responsive to the respective broad or actively managed portfolio and shall include but not be limited to research on:

- **Improving women Veteran's health and healthcare across the lifespan and across services**, including primary care/prevention, reproductive health, intimate partner violence, military sexual trauma, traumatic brain injury (TBI), mental health, substance and alcohol use disorders, response to pain and opioid use, suicide prevention (including lethal means safety), complex chronic conditions and multimorbidity, long-term care, aging, cancer care, cardiovascular disease, chronic kidney disease, autoimmune diseases, menopause and perimenopause, endometriosis and fibroids, Alzheimer's disease, rural health, LGBTQ+ veteran health, and more.
- **Health issues and conditions that affect women Veterans uniquely, as well as those affecting women disproportionately or differently from men**, including those related to race, ethnicity, age, socioeconomic status, disability, sexual orientation or gender identity, and military service and experience.
- **Health care needs and interventions for post-deployment health that account for military experiences and exposures**, such as transition from active duty to Veteran status, military-related trauma, injuries such as TBI, exposure to environmental factors and contaminants that can directly affect health.
- **Health and healthcare for midlife and older women Veterans**, including aging, menopause, and long-term care, as well as women Veterans' mental health and reproductive mental health during midlife (during perimenopause and early menopause).
- **Translational research and evidence across the lifespan and across services** (e.g., from basic to clinical to health systems to implementation in routine practice).

In addition to these overarching women's health research priorities, the broad portfolios also have the following specific areas of interest:

Health Systems Research Priorities:

- Research on the health issues referenced above that assess Quintuple Aim goals (improve Veteran outcomes, access equity, experience, and value) using learning health

system methods including implementation science, data science, engagement science, systems science, and policy science.

- Research related to healthcare access and community care, such as:
 - Coordination of community care women's health services, community-provided pregnancy and post-partum care, and care for complex health conditions
 - Quality of care received in the community vs direct care, as well as difference in practice patterns.
- Strategies and interventions for implementing, spreading, and sustaining evidence-based practices that improve women Veterans' care and outcomes, e.g., to enhance trauma-informed care, end harassment and intimate partner violence, and to provide gender-tailored interventions where needed.

Rehabilitation Research, Development and Translation Priorities:

- Research on the health issues referenced above and studies that include aims addressing:
 - Health disparities and conditions that impact underserved Veterans including, but not limited to, Veterans of racial, ethnic, sex, gender, or LGBTQ+ minority populations or Veterans with disabilities, homelessness experienced, or justice system involved. All within the context of understanding onset, severity, duration, rehabilitation, and recovery from disability.
 - Prosthetic and other assistive technology needs of women Veterans.

Medical Health Research Priorities:

- Research that focuses on conditions that affect women Veterans **uniquely** (e.g., breast and gynecological cancers, menopause, endometriosis, uterine fibroids, fertility, reproductive health, etc.), **disproportionately** (e.g., autoimmune diseases, arthritis, chronic kidney disease, etc.), **differently** (e.g., cardiovascular disease, etc.), and conditions **related to military service** (e.g., military exposures).
- Studies need not be specific to women but must advance knowledge of women's health (e.g., sex or gender-based analysis appropriately powered to report stratified outcomes or findings).
- Translational research and the development of innovative strategies and therapies to improve the health of women Veterans.
- Projects that address sex-based differences including the use of female animals to study sex as a biological variable and biomedical studies addressing sexual dimorphism of diseases and conditions in women.
- In vivo and in vitro studies of female hormones or projects that focus on Veterans identifying as women, gender-affirming therapies for Veterans assigned female at birth, or transgender women, and complementary studies conducted in animals to better understand the effects of transgender therapies.

Brain, Behavioral and Mental Health Research Priorities:

Research on health issues referenced above and others that address the complex needs of women Veterans in the areas of whole brain health, behavioral health, and mental health.

Process for Application Submission and Review: Prior to the submission of a full research application, a Pre-application (I02) must be submitted and approved by the Women's Health leadership.

Submitting a Pre-application (I02) to Grants.gov/eRA (including any waiver requests [budget, duration, eligibility, full offsite]) is the first step in the application process.

For current ISRM RFAs go to: <https://vawww.research.va.gov/funding/rfa.cfm>

NOTE: All eligibility, Pre-application and companion full research application requirements and submission deadlines detailed in the published ISRM RFAs as well as content/format requirements outlined in the [VA-ORD SF 424 Application Guide R&R](#) must be met.

Agency Contacts: VA-ORD encourages scientific/programmatic inquiries concerning this NOSI and welcomes the opportunity to answer any questions.

For general review questions/issues: vhalSRM@va.gov

For questions about the Women's Health and Healthcare:

Amanda Borsky, Ph.D.: Amanda.Borsky@va.gov

Grants.gov or eRA Commons questions/issues: vhaacordera.vhaacordera@va.gov

ⁱ U.S. Department of Veterans Affairs. (2022). Facts and Statistics, Women Veterans in Focus.

<https://www.womenshealth.va.gov/WOMENSHEALTH/materials-and-resources/facts-and-statistics.asp>

ⁱⁱ Frayne SM, Phibbs CS, Saechao F, Friedman SA, Shaw JG, Romodan Y, Berg E, Lee J, Ananth L, Iqbal S, Hayes PM, Haskell S. Sourcebook: Women Veterans in the Veterans Health Administration. Volume 4: Longitudinal Trends in Sociodemographics, Utilization, Health Profile, and Geographic Distribution. Women's Health Evaluation Initiative, Women's Health Services, Veterans Health Administration, Department of Veterans Affairs, Washington DC. February 2018.

ⁱⁱⁱ Sheahan, K. L., Goldstein, K. M., Than, C. T., Bean-Mayberry, B., Chanfreau, C. C., Gerber, M. R., Rose, D. E., Brunner, J., Canelo, I. A., Darling MSHS, J. E., Haskell, S., Hamilton, A. B., & Yano, E. M. (2022). Women Veterans' Healthcare Needs, Utilization, and Preferences in Veterans Affairs Primary Care Settings. *Journal of General Internal Medicine*, 37(S3), 791–798. <https://doi.org/10.1007/s11606-022-07585-3>